

DIABETIC RETINOPATHY ANALYSIS

AI-Powered Retinal Assessment Report

Report Date:	27-07-2026	Report ID:	F82CA3B7-958
Prediction:	Moderate DR	Analysis Time:	17:56:49

Clinical Interpretation

- Retinal imaging reveals multiple scattered microaneurysms and numerous dot-and-blot hemorrhages distributed across several quadrants of both fundi
- signifying moderate non-proliferative diabetic retinopathy. Crucially
- no signs of intraretinal microvascular abnormalities (IRMA)
- venous beading
- or neovascularization were identified at this evaluation.

Disease Summary

- Moderate diabetic retinopathy indicates an advancement in the retinal vascular damage caused by diabetes
- characterized by more widespread leakage and bleeding within the retina compared to mild stages. This stage confirms progressive microvascular changes but
- importantly
- has not yet developed the new
- fragile blood vessels seen in proliferative disease.

Possible Medical Concerns

- The primary concern at this stage is the potential for progression to severe non-proliferative or even proliferative diabetic retinopathy
- which carries a higher risk of vision-threatening complications. There is also an ongoing risk of developing diabetic macular edema (DME)
- which can lead to central vision loss and requires timely intervention.

Treatment Guidance

- Management at this juncture hinges critically on optimizing systemic control of blood glucose levels
- blood pressure
- and lipid profiles
- in close collaboration with your endocrinologist or primary care physician. Direct ocular intervention such as laser therapy or anti-VEGF injections is not typically indicated for moderate NPDR unless significant macular edema concurrently develops.

Lifestyle Recommendations

- Strict adherence to your prescribed diabetic dietary regimen
- consistent engagement in moderate physical activity
- and maintaining a healthy body weight are paramount to slowing disease progression. Furthermore
- cessation of smoking
- if applicable
- is strongly encouraged to preserve retinal vascular integrity and overall health.

Follow-up Advice

- A diligent follow-up comprehensive ophthalmic examination is strongly recommended within 6 to 9 months to monitor for any signs of progression or the development of treatable complications like macular edema. Promptly report any new visual disturbances
- such as blurred vision
- floaters
- or vision loss
- to your ophthalmologist.

Medical Disclaimer

- This report provides a professional medical assessment based solely on the retinal imaging submitted and does not constitute a doctor-patient relationship. It is not a substitute for a thorough clinical examination or personalized medical advice from your treating physician, who should be consulted for all treatment decisions and health concerns.